Which treatment this guide covers NCCP 00219a is first-line EGFR-mutated advanced NSCLC; 00219b is switch-maintenance after stable disease following first-line chemotherapy; 00219c is after failure of at least one previous chemotherapy regimen. Global NCCP eligibility requires a validated activating EGFR mutation in each setting. These are advanced-disease settings, not adjuvant treatment.

NCCP 00219 · VERSION 6

Erlotinib monotherapy

The usual dose is 150 mg by mouth once daily, continuously without a planned rest week. Use the dose on your prescription. Treatment continues while it helps and side effects remain acceptable; any change is agreed with your team.

BASELINE → TREATMENT → FOLLOW-UP

Treatment and monitoring over time

Relative stages, not equal lengths of time or booked appointments. Daily tablets continue between reviews unless the team tells you to hold. Confirm who arranges and reviews every test.

Check	Before the first dose	Early treatment review	Ongoing daily treatment	Benefit / toxicity review	Treatment change or completion
Treatment	Confirm indication and prescribed dose	Daily erlotinib; no planned rest week	Continue the prescribed dose	Review benefit, harms and dose	Agree next treatment or follow-up
Blood tests	Blood count, kidney and liver profile	Every 60 days; earlier if clinically needed	Continue every 60 days	Extra tests for dehydration or liver problems	Follow unresolved abnormalities
Symptoms and medicines	Review lungs, bowel, skin, eyes and medicines	Report bowel, skin or eye symptoms early	Check adherence, supply and interactions	Confirm recovery before restart	Continue advice for unresolved symptoms
Cancer assessment	Confirm EGFR, stage and prior treatment	Clinical review	Scans per Consultant plan	Discuss response or progression	Agree follow-up or next treatment

THE WHOLE TREATMENT JOURNEY

Erlotinib monotherapy: treatment and checks

Activating-EGFR advanced NSCLC

The usual dose is 150 mg by mouth once daily, continuously without a planned rest week. Use the dose on your prescription. Treatment continues while it helps and side effects remain acceptable; any change is agreed with your team.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before the first dose	Early treatment review	Ongoing daily treatment	Benefit / toxicity review	Treatment change or completion
Treatment NCCP 00219 · SMPC	Confirm indication and prescribed dose	Daily erlotinib; no planned rest week	Continue the prescribed dose	Review benefit, harms and dose	Agree next treatment or follow-up
Blood tests NCCP 00219 · SMPC	Blood count, kidney and liver profile	Every 60 days; earlier if clinically needed	Continue every 60 days	Extra tests for dehydration or liver problems	Follow unresolved abnormalities
Symptoms and medicines NCCP 00219 · SMPC	Review lungs, bowel, skin, eyes and medicines	Report bowel, skin or eye symptoms early	Check adherence, supply and interactions	Confirm recovery before restart	Continue advice for unresolved symptoms
Cancer assessment NCCP 00219 · SMPC	Confirm EGFR, stage and prior treatment	Clinical review	Scans per Consultant plan	Discuss response or progression	Agree follow-up or next treatment

What each check is for

1 Confirm eligibility and the prescription

Before the first dose

The mutation and treatment setting determine applicability.

Ask / act: Document exact indication, dose and initial investigations.

Sources: NCCP 00219

2 Early bowel and skin review

From the first dose and early reviews

Early support can prevent dehydration and uncontrolled skin toxicity.

Ask / act: Record grade, duration, supportive treatment and recovery; apply medicine-specific thresholds.

Sources: NCCP 00219 · SMPC

3 New respiratory symptoms

At any time

Possible ILD requires urgent investigation.

Ask / act: Arrange urgent assessment and an authorised medicine plan.

Sources: NCCP 00219 · SMPC

4 Benefit and next-plan review

At planned cancer assessments or significant toxicity

Continuous treatment is not an unlimited automatic prescription.

Ask / act: Agree continuation, hold, restart, change or follow-up.

Sources: NCCP 00219

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

SACTCheck · Developed by Dr Paul O'Brien · v0.84.26 · Sources checked 2026-10-29
Source reconciled; independent clinical, pharmacy and patient/PPI review pending